Based Treatment (MBT; Bateman and Fonagy 2012), Transference-Focused Psychotherapy (TFP; Yeomans et al. 2002), and Schema-Focused Therapy (SFT; Young 1990). These evidence-based treatments are intended for professionals who self-select because they want to develop expertise (i.e., they want to specialize in treating, and especially in doing extended individual psychotherapy for, BPD). For these evidence-based treatments, patients are expected to attend regular sessions, usually for 2 or more hours per week for a year or more. These treatments can be adapted for specialized BPD clinical services at tertiary facilities such as McLean Hospital (www.mclean.harvard.edu). Because of the extensive training required to attain competence in these evidence-based BPD-specific therapies and because of the commitment in time and costs they require, these therapies are rarely available. Given the estimated 2% community prevalence of BPD (i.e., in the United States, that means about 6 million people), the BPD-specific psychotherapies cannot be expected to meet the needs for their treatment. (See Appendix A, "Relation of Good Psychiatric Management to Other Evidence-Based Treatments for Borderline Personality Disorder," for further consideration of how GPM compares with these other evidence-based treatments.)

GPM offers a more basic approach to treatment of BPD than do prior evidence-based therapies, it is more suitable for initial "entry-level" training, and it contains what every primary BPD treater should know. It clearly and directly includes the perspectives of a genetic disposition and an expectation of improvement. This approach can be integrated into the standardized training curriculum by departments of psychiatry or psychology. Those patients who fail to respond to GPM or who—having been stabilized by GPM—still want better self-regulation or self-awareness are good candidates for the more intensive and extended evidence-based treatments cited above. (Such an instance is described in Chapter 8 ["Case Illustrations"]: Case 5, Lawrence.) Although these prior BPD-specific psychotherapies dramatically differ in theory and interventions, they are all comparably effective (Gabbard 2007). This conclusion has drawn attention to the therapeutic value of their common features (see Appendix B, "Common Features of Evidence-Based Treatments for Borderline Personality Disorder"). Because GPM emphasizes these common features, it will often prove capable of producing the same basic changes effected by its more rigorously defined and austerely constraining predecessors.

GPM's Precedents and Foundations

There have been many precedents from which GPM has drawn. Most notably, it is drawn from the first author's lifetime of experience in treating

BPD—experience previously documented in a series of books (Gunderson 1984, 2001; Gunderson and Links 2008). Even the first of these books (Gunderson 1984) described the basic practicalities of managing patients with BPD across modalities and levels of care as well as introducing the interpersonal focus that remains constant in GPM. Others have pioneered advocacy for the long-underappreciated value of supportive (Rockland 1987, 1992) and pragmatic (Dawson and MacMillan 1993) interventions in the treatment of BPD. GPM proudly integrates those perspectives.

Longitudinal research showing frequent, dramatic, and enduring improvements in borderline patients who did *not* receive extended or intensive therapies, much less evidence-based BPD-specific treatments, offers convincing evidence that short-term or intermittent treatment interventions can have enduring benefits (Gunderson et al. 2011; Zanarini et al. 2010). Particularly relevant was the finding that a significant number remitted within 6 months with the help of brief interventions (Gunderson et al. 2003). Two decades ago after his seminal longitudinal research, McGlashan (1993) reached a similar conclusion that borderline patients can benefit from short-term, intermittent, as-needed interventions. Notably, this research also documented the relatively poor outcomes of BPD patients in the areas of work and stable partnerships, thereby drawing attention to an area of outcome largely unaddressed by BPD's established treatments.

The randomized controlled trials that have documented the value of BPD-specific psychotherapies also lend strength to GPM's claims for effectiveness. These studies originally used weak, unregulated "treatment as usual" comparators, but in three recent trials, comparison treatments that have performed very well are markedly GPM-like. Specifically, GPM can claim marked similarities to the Expressive-Supportive (Clarkin et al. 2007), Structured Clinical Management (Bateman and Fonagy 2009), and Good Clinical Care (Chanen et al. 2008) treatments, which each did as well as TFP, MBT, and Cognitive Analytic Therapy (CAT), respectively, in almost every area of outcome. These results yielded further support for our thesis; GPM-like control treatments can do as well as their more celebrated index treatment comparators.

As shown in Table 1–1, GPM is a treatment model that is quite distinct from other evidence-based treatments by virtue of its emphasis on the interpersonal sources of emotional and behavioral symptoms and its use of psychoeducation (including discussions of genetic contributions and expectable change) and because it actively and flexibly encourages multimodal interventions. It also has the advantages of having incorporated concepts, approaches, and specific forms of intervention emphasized within other treatment models (see Table 1–2). In this sense, it is a second-generation treatment model that has selectively pruned and adapted contributions by

TABLE 1–1. Distinctive characteristics of GPM

Element	Distinctive features
Case management	Focus is on the patient's life outside therapy, not primarily on the patient's psychology as in psychotherapies.
Psychoeducation	Patients and families are informed of borderline personality disorder's genetic disposition, expectable changes, and the relative merits of different approaches.
Goals	Symptom reduction and self-control are secondary goals required to attain the primary goals of success in work and partnerships.
Multimodality	Psychopharmacological practices are integrated as adjunctive alongside endorsements for group therapies and family coaching.
Duration and intensity	No specific length and intensity are prescribed; patients and therapist collaborate in judging whether a therapy is effective.
Interpersonal hypersensitivity	An explicit and consistent effort is made to connect the patient's emotions and behaviors to interpersonal stressors.

others, hopefully into a more pragmatic, generalizable, and user-friendly model.

In 2001, the "Practice Guideline for the Treatment of Patients With Borderline Personality Disorder" developed by the American Psychiatric Association (American Psychiatric Association Practice Guidelines 2001) offered a summary that overlaps with this handbook's content and intent. GPM offers considerably more emphasis on case management and less emphasis on individual psychotherapy than did the American Psychiatric Association guidelines. This handbook updates information included in a previous book by Gunderson and Links (2008) but does not purport to be as comprehensive. It adds empirically validated specificity about interventions within a specific (i.e., interpersonal hypersensitivity) model. Most significantly, this book includes two types of case-based material to facilitate training. The first is a series of case illustrations with discussions about the variety of responses clinicians might deploy in managing BPD. The second is a set of online videotaped interventions. In general, this book serves a function and proposes a treatment approach that overlaps with many precedents but remains unique and original.

TABLE 1–2. GPM's integration of prior evidence-based psychotherapies

Modality	Element
Transference-Focused Psychotherapy	
Concepts	Splitting and projection as defenses against anger
Therapeutic stance	Monitoring boundaries and countertransference
Interventions	Interpretation of anger; challenge avoidance and acting out
Dialectical Behavior Therapy	
Concepts	Social and psychological skill deficits
Therapeutic stance	Coaching
Interventions	Self-monitoring; homework; intersession availability
Mentalization-Based Treatment	
Concepts	Theory of mind; attachment
Therapeutic stance	Not-knowing
Interventions	Examining attributions about self and others

GPM's Empirical Validation

In 2009, McMain et al. published a report of a large multisite randomized controlled trial showing that the clinical efficacy of General Psychiatric Management equaled that of DBT.

The "General" Psychiatric Management used in that trial can be considered the research "brand name" for the "Good" Psychiatric Management described in this book. The efficacy of General Psychiatric Management found in that study justifies the name change used here. GPM combined a psychodynamically informed case management based on the authors' previous book, *Borderline Personality Disorder: A Clinical Guide* (Gunderson and Links 2008), with the symptom-targeted medication algorithm suggested in the American Psychiatric Association Practice Guidelines (2001). This was a large single-blind multisite trial in which 180 patients diagnosed with borderline personality disorder who had at least two suicidal or nonsuicidal self-injurious episodes in the past 5 years were randomly assigned to receive 1 year of either DBT or GPM. Therapists in the GPM cell were psychiatrists

(there was one psychologist) with aptitude, interest, and an average of 5 years' experience working with borderline patients. They met with the project's coinvestigator (Paul S. Links, M.D., M.Sc., F.R.C.P.C.) weekly for 6 months before the project to discuss GPM's principles and then for weekly peer supervision for the duration of the trial. Therapists in the DBT cell were primarily psychologists with prior training in DBT who met weekly with the project's Principal Investigator, Shelley McMain, as their consultation group leader throughout the project. Adherence to both treatments was established and sustained.

Both groups showed similar levels of improvement on a variety of clinical outcome measures, including significant reductions in the frequency and severity of suicidal and nonsuicidal self-injurious episodes; significant reductions in health care use, including emergency department visits and psychiatric hospital days; and significant improvements in BPD symptoms, symptom distress, depression, anger, and interpersonal functioning. No significant difference across any of these outcomes was found between the two treatments. A 2-year follow-up showed that patients sustained the gains made on suicidal and other self-injurious behaviors, health care use, and psychopathology (McMain et al. 2012). At 3 years, 62% of the patients in both groups had remitted to fewer than two criteria for a year or more, a rate that was 20% higher than for borderline patients who did not receive these BPD-specific therapies (Gunderson et al. 2011). These results indicated that a moderately well-specified treatment delivered by experienced psychiatrists who were guided by the principles described in this handbook could be highly effective—as effective as was treatment with high-quality DBT.

Although the *General* Psychiatric Management manual used during that trial rested heavily on *Borderline Personality Disorder: A Clinical Guide* (Gunderson and Links 2008), some distinctions are found between the General Psychiatric Management conducted in that trial and the *Good* Psychiatric Management guidelines offered here. One difference is that the "General" research brand was designed to be 1 year long to be comparable to DBT and thereby required informed consent involving the expectation of a 1-year commitment. The duration of "Good" Psychiatric Management as described here is pragmatic—as long as useful. Moreover, the General Psychiatric Management therapists were already experienced and liked working with borderline patients. This handbook reflects some of what those experienced psychiatrists had already learned to do, but it is intended to help less experienced treaters gain that level of competence more quickly. The third and potentially more substantive difference is that the General Psychiatric Management brand gave more hierarchical emphasis to self-harm and processing emotion than does this handbook. Those emphases may have diminished the distinctions from DBT. The model used for the

"General" research brand was that suicidal and other self-harming behaviors were mediated by problems with emotional control or processing. GPM treaters thus attempted to enhance awareness, identification, and acceptance of feeling states. The GPM described in this handbook has these same goals but ties the control of both self-harm and emotions to interpersonal stressors and to the mediating role of having a sustained and trustworthy ally via the GPM treater. A fourth difference is that "Good" PM gives a more explicit emphasis to the personal accountability, and to the goals of achieving both vocational satisfaction and stable partnerships, than did the General Psychiatric Management manual. How the study's psychiatrists actually managed this is unknown. It was clear that the "General" research brand, like all other evidence-based therapies for BPD, was not particularly effective in social or vocational rehabilitation. It is hoped that the added emphasis given here might improve that area of outcome. Finally, whereas the General Psychiatric Management brand used the American Psychiatric Association Practice Guidelines (2001) algorithm as a framework for medications (although clinicians participating in the research trial frequently departed from that algorithm on the basis of post-2001 research), in this handbook we formally propose an updated algorithm that integrates subsequent research. It is unclear whether and how any or all of these differences are significant, but, for clarity, they are highlighted here.

The results from the McMain et al. (2009) study endorse the idea that well-informed clinicians without the extensive BPD-specific training required for DBT, MBT, TFP, CAT, or SFT can provide excellent care. The research-based endorsement of GPM underscores the hope that many psychiatrists and other mental health professionals who now greet the prospect of treating BPD with unrealistic fears about their ability to be helpful can become self-confident and effective treaters.

SECTION II

GPM Manual

Treatment Guidelines